THE ALLIANCE

Official Comment: Request for Information Regarding Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care

The Alliance submits this formal response to the Request for Information regarding the adoption and use of artificial intelligence in clinical care. Our commentary is derived from the aggregated experiences and preferences of our ~100 members who are patients, caregivers, and everyday citizens: the “end users” of American healthcare. About 5 members are citizens of other countries, whom we asked to share their perspectives as well, as we believed they would possess valuable comparative perspectives.

The open-ended survey questions we shared with members can be found in an appendix below. All analysis of the survey was completed by members of the Alliance Strategic Office. All member responses were manually reviewed.

I. Patient Trust and Barriers to Adoption (Addressing Questions 1 and 9)

In response to Question 1 regarding barriers to adoption and Question 9 regarding the concerns of patients: the Department must recognize that **public trust in medical AI is highly conditional on human oversight, transparency, bias mitigation, and other such factors**.

When surveyed on how their opinion of their physician would change if they learned the doctor used AI tools to make clinical decisions, 60.0% of Alliance members stated their opinion would stay the same or depend heavily on the conditions of its use. However, 25.9% reported their opinion would actively decrease, while only 11.8% said it would increase.

The primary barrier to patient trust is the fear of clinical degradation. Members expressed **deep concern that AI use would displace independent clinical judgment**. One member wrote: "If they are just using [it] as a sort of encyclopedia to jog the memory, I would take little offense... but if they did not validate the assertions made by the AI I would be highly skeptical." Alliance members largely rejected AI as an autonomous decision-maker; they expect it to be used strictly as an assistive tool and authority and accountability to remain tied to a human physician.

Additionally, a large majority of Alliance members indicated they tolerate mistakes made by AI less than mistakes made by humans. **Many cited a belief that AI would not be held accountable the way that humans are.**

II. Administrative Efficiency as the Primary Goal (Addressing Questions 2, 6, and 7)

In response to Question 7 regarding administrative hurdles and Question 2 regarding programmatic and payment policy changes: **the Department should prioritize the adoption of AI tools that alleviate administrative burnout.**

When asked what challenges they wish to see addressed by AI, Alliance members **consistently mentioned the bureaucratic inefficiencies of modern healthcare.** Alliance members want AI to handle tasks such as charting, automated dictation, scheduling, and insurance eligibility verification. By using AI to increase administrative efficiency, the Department can allow healthcare professionals to spend more time with their patients. One member with clinical work experience stated: "I would like AI to address the challenges of administrative burnout, by automating paperwork and scheduling so that doctors can spend more face-to-face time with their patients."

III. Bridging Disparities and Enhancing Diagnostics (Addressing Questions 6 and 9)

In response to Question 6 regarding where novel AI tools have the greatest potential: **Alliance member responses indicate strong support for AI when deployed to correct human shortcomings**, specifically in areas of diagnostic delay and medical bias.

Patients view AI as a highly promising **"Second Opinion Framework."** Alliance members highlighted the potential for AI to act as a "neutral" arbiter to combat systemic medical biases, noting that women and minority populations have historically experienced delayed diagnoses or had their symptoms minimized due to said biases. A properly trained AI could flag these disparities and ensure symptoms are evaluated objectively.

Furthermore, members **strongly support the use of AI in pattern recognition and image analysis** (e.g., X-rays, MRIs) and for diagnosing "long tail" or rare diseases that a standard practitioner might only see once in a career. However, such usage requires AI that is transparent about how it reaches its conclusions and is demonstrably accurate, so clinicians and patients can trust that its recommendations are reliable and unbiased.

Sincerely,

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Appendix A: Survey questions

These are the questions we asked to Alliance members

Question 1: If you learned your doctor was using AI tools to help make clinical decisions, would your opinion of their care increase, decrease, or stay the same? Why?

Question 2: What challenges within healthcare do you wish to see addressed by the adoption and use of AI in clinical care?

Question 3: Are you more or less willing to tolerate errors from AI systems than you are from humans? Why?